

Dexa

0.5% w/v
Solution for injection

COMPOSITION

Dexamethasone sodium phosphate.....5 mg
Exp. qs1 ml

DESCRIPTION: The finished product is colorless transparent solution

PHARMACODYNAMICS

This preparation contains the sodium phosphate ester of dexamethasone, a fluoro-methyl derivative of prednisolone, which is a potent glucocorticoid with minimal mineralocorticoid activity. Dexamethasone has ten to twenty times the anti-inflammatory activity of prednisolone. Following intramuscular injection this soluble ester of dexamethasone is rapidly absorbed and hydrolysed to the parent alcohol giving a prompt response which is maintained for approximately 48 hours.

PHARMACOKINETICS

Dexa 0.5% is a short acting dexamethasone preparation with a rapid onset of activity. It contains the disodium phosphate ester of dexamethasone. After extravascular (intramuscular, subcutaneous, intra-articular) administration, the ester is rapidly resorbed from the injection site followed by immediate hydrolysis into the parent compound, dexamethasone. Absorption of dexamethasone is rapid. Bioavailability following i.m. administration (compared to i.v. administration) is high in all species. After intramuscular administration, apparent elimination half-life has been shown to range between 1 and 20 hours according to the species.

INDICATION

The product is indicated as an anti-inflammatory agent in dogs and cats. Small animal indications include inflammatory conditions involving the joints (where structural changes such as ankylosing joints, ruptured ligaments, sheaths, etc. do not exist) and nonspecific dermatitis.

TARGET SPECIES

CATS & DOGS

RECOMMENDED DOSE

Therapy with Dexa 0.5% should be individualised according to the severity of the condition being treated, the anticipated duration of steroid therapy and the patient's threshold or tolerance for steroid excess. The lowest dose that will offer adequate relief in chronic conditions should be the dose employed. Large doses, however, may be necessary and in such instance, the patient must be closely observed for the occurrence of side effects. Acute conditions sometimes demand the use of larger doses for a short period of time in order to obtain the necessary degree of relief. When side effects occur, it may be necessary to reduce the dose of the drug or to discontinue therapy. Administer intravenously or intramuscularly.

DOGS: 0.25 to 1.0 mg I.V. or I.M. The dose may be repeated if necessary.

CATS: 0.125 to 0.5 mg I.V. or I.M. The dose may be repeated if necessary.

When using Dexa 0.5% as supportive therapy in dogs and cats, the doses suggested above may be used. The use of this product for the above mentioned conditions does not preclude the necessity for using complementary therapy to treat the primary condition.

ROUTE OF ADMINISTRATION

Intravenous or intramuscular

CONTRA-INDICATION

Except in emergency situations the product should not be used in animals suffering from diabetes, chronic nephritis, renal disease, congestive heart failure, osteoporosis and in viral infections during the viraemic stage.

INTERACTION WITH OTHER MEDICINAL PRODUCTS

Because corticosteroids can reduce the immuno response to vaccination, the product should not be used in combination with vaccines.

WARNINGS AND PRECAUTIONS

All of the precautions and contraindications for adrenocortical hormones must be followed, taking into consideration that the warning signs of cortisone overdosage such as sodium retention, fluid retention, potassium loss, weight gain, etc. may not be present. Animals receiving the drug should be under close observation to detect any possible untoward effects.

One should consider the possibility that continuous large doses of this product will result in some depression of adrenocortical function and may produce adrenal atrophy.

After long-term therapy, the drug should be withdrawn gradually rather than abruptly. As with all glucocorticoids, this product may mask the signs of infection or cause the onset of latent infections. For these reasons, it may be advisable to institute proper antibacterial therapy together with this product. The possible action of this product in delaying wound healing should also be kept in mind. This product is a synthetic analogue of prednisolone having similar, but more potent, anti-inflammatory therapeutic action and diversified hormonal and metabolic effects.

As with any drug, animals receiving this product should be kept under close observation and the dosages recommended should not be exceeded. The dosage of this product required is markedly lower than that of prednisolone. The sections on indications, dosage and precautions should be read before using this drug. Clinical and experimental data have demonstrated that corticosteroids administered parenterally to animals may induce the first stage of parturition when administered during the last stage of pregnancy and may precipitate premature parturition followed by dystocia, fetal death, retained placenta and metritis.

PREGNANCY AND LACTATION

corticosteroids are not recommended for use in pregnant animals. Administration in early pregnancy is known to have caused foetal abnormalities in laboratory animals. Administration in late pregnancy may cause early parturition or abortion.

SIDE EFFECTS

Steroids themselves, during treatment, may cause Cushingoid symptoms involving significant alteration of fat, carbohydrate, protein and mineral metabolism, e.g. redistribution of body fat, muscle weakness and wastage and osteoporosis may result. During therapy effective doses suppress the hypothalamo-pituitary-adrenal axis. Following cessation of treatment, symptoms of ad-reanal insufficiency extending to adrenocortical atrophy can arise and this may render the animal unable to deal adequately with stressful situations. Consideration should therefore be given to means of minimising problems of adrenal insufficiency following the withdrawal of treatment, e.g. dosing to coincide with the time of the endogenous cortisol peak (i.e. in the morning with regard to dogs and the evening re cats) and a gradual reduction of dosage (for further discussion see standard texts). Systematically administered corticosteroids may cause polyuria, polydipsia and polyphagia, particularly during the early stages of therapy. Some corticosteroids may cause sodium and water retention and hypokalaemia in long term use. Systemic corticosteroids have caused deposition of calcium in the skin (calcinosis cutis). corticosteroids are not recommended for use in pregnant animals. Administration in early pregnancy is known to have caused foetal abnormalities in laboratory animals. Administration in late pregnancy may cause early parturition or abortion. Corticosteroids may delay wound healing and the immunosuppressant actions may weaken resistance to or exacerbate existing infections. In the presence of bacterial infection, antibacterial drug cover is usually required when steroids are used. In the presence of viral infections, steroids may worsen or hasten the progress of the disease.

Gastro-intestinal ulceration has been reported in animals treated with corticosteroids and gastro-intestinal ulceration may be exacerbated by steroids in patients given non-steroidal anti-inflammatory drugs and in animals with spinal cord trauma. Steroids may cause enlargement of the liver (hepatomegaly) with increased serum hepatic enzymes.

In very rare cases (less than 1 animal in 10,000 animals, including isolated reports), hypersensitivity reactions might occur

SYMPTOMS AND TREATMENT OF OVERDOSE

See side effects section

STORAGE CONDITION

Store at temperature not exceeding 30°C. Strictly keep away from sunlight and avoid heat.

SHELF LIFE

2 years

SHELF LIFE AFTER OPENING

24 hours

PACKAGING

100ml

REVISED DATE

Jun 24, 2019